

Patient Results

M

Surgical Pathology (Copath)

Final

Path Report

Final

SURGICAL PATHOLOGY REPORT

ACCESSION NO. : [REDACTED]

Final Diagnosis(es):

- (A) Soft tissue, left central neck, biopsy: Benign soft tissue.
- (B) Soft tissue, left central neck, biopsy: Parathyroid gland, normocellular.
- (C) Lymph nodes, left central neck, level 6, dissection:
- 1) Four lymph nodes positive for metastatic papillary carcinoma (4/4).
 - 2) Extracapsular extension present.
- (D) Thyroid, total thyroidectomy:
- 1) Papillary thyroid carcinoma (see synoptic report).
 - 2) Left surgical margin positive
 - 3) Extrathyroid extension present.
- (E) Soft tissue, right central neck, biopsy:
- 1) Parathyroid tissue, normocellular.
 - 2) Fibrotic tissue.

Synoptic Report:

Procedure: Total thyroidectomy with central compartment dissection.

Received: In formalin.

Specimen Size:

Right lobe: 4.6 x 3.2 x 1.8 cm.

Left lobe: 5.0 x 2.8 x 1.3 cm.

Isthmus: 3.7 x 2.3 x 2.2 cm.

Specimen Weight: 28.1 gm

Specimen Integrity: Intact.

Tumor Focality: Multifocal, bilateral.

Dominant Tumor:

Tumor Laterality: Isthmus.

Histologic Type, Dominant: Papillary carcinoma.

Variant: Classical (usual)

Architecture: Classical (papillary)

Cytomorphology: Classical

Tumor Size, Dominant:

Greatest dimension: 2.1 cm

Additional dimensions: 1.5 x 1.5 cm.

Margins: Margins involved by carcinoma.

Sites of involvement: Left side.

Encapsulation: Partially encapsulated.

Capsular Invasion: Present, widely invaded.

Lymphovascular Invasion: Not identified.

Extrathyroid Extension: Present, minimal.

Pathologic Stage:

Primary Tumor: pT3: Tumor more than 4 cm limited to thyroid or any tumor with minimal extrathyroid extension.

1CD-0-3

carcinoma, pap. thy, thyroid
826013

Site: thyroid, nos c73.9

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Regional Lymph Nodes: pN1a Nodal metastases to level VI lymph nodes.

Lymph node, extranodal extension: Present.

Distant metastasis: Not applicable.

[REDACTED]
The gross description and all microscopic slides have been reviewed and interpreted by the undersigned pathologist'

Electronically Signed

[REDACTED] (Resident)

Specimen(s) Received:

A: Left central neck nodule

B: Left central neck nodule

C: Left central neck nodules, level 6

D: Thyroid and isthmus

E: Right central neck lymph node

Clinical History:

Papillary thyroid cancer. Total thyroidectomy, neck dissection. Tumor Bank.

(A) Confirm parathyroid.

(B) Confirm parathyroid.

(D) Short superior, long left superior.

[REDACTED]
Intraoperative Consultation:

Time Received:

Time Reported:

FSA1: Left central neck nodule: Defer to permanent (1 block).

FSB1: Left central neck nodule: Benign parathyroid tissue (1 block).

[REDACTED]
[REDACTED] (Resident)

Gross Description:

(A) (left central neck nodule) Received fresh for frozen section is a tan-yellow nodule measuring 0.5 x 0.3 x 0.2 cm. The specimen is entirely submitted for frozen section following which is submitted for permanent in FSA1.

(B) (left central neck nodule) Received fresh for frozen section is a tan-pink nodule measuring 0.2 x 0.2 x 0.2 cm. The specimen is entirely submitted for frozen section following which is submitted for permanent in FSB1.

(C) (left central neck nodules, level 6) Received in formalin is a tan-yellow to black mass measuring in aggregate 3.0 x 2.4 x 2.0 cm. There are six potential lymph nodes measuring from 0.3 to 1.7 cm in diameter. Transection of the largest lymph node reveals a cut surface that is pale to black and soft. The specimen is entirely submitted in C1. A second large lymph node is bisected, revealing a pale tan inside with a black rim around the capsule. The specimen is entirely submitted in C2. Four smaller potential lymph nodes are submitted in C3.

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(D) (thyroid and Isthmus) Received fresh for sampling is a total thyroidectomy.

Specimen components and dimensions: Right lobe is 4.6 x 3.2 x 1.8 cm, weighing 10.6 gm. Left lobe is 5.0 x 2.8 x 1.3 cm, weighing 10.3 gm. The isthmus is 3.7 x 2.3 x 2.2 cm, weighing 7.2 gm.

Inking Scheme: The specimen is inked as follows: right black, left blue, isthmus red.

Size, appearance, and location of tumor: Left lobe demonstrates three nodules, the first measuring 0.6 x 0.4 x 0.3 cm, composed of pale tan fibrous tissue. Second nodule is 0.8 x 0.7 x 0.7 cm and is surrounded by a white calcified wall. Tumor itself is pale friable material. Third nodule is 0.6 x 0.4 x 0.3 cm and is tan to white. The isthmus is almost entirely replaced with one tumor nodule measuring 2.1 x 1.5 x 1.5 cm. It is composed of tan-yellow friable material. There are areas of calcification within the isthmus. Number of tumor nodules: Right lobe demonstrates two nodules. The first nodule is white tan, calcified and measures 1.1 x 0.5 x 0.5 cm. The second nodule measures 0.9 x 0.4 x 0.3 cm, is tan-yellow with a white fibrous wall encapsulating it and is calcified.

Capsular penetration: Tumors in the left, right lobe and isthmus are abutting the capsule.

Uninvolved parenchyma: Uninvolved parenchyma is red-brown and soft.

Blocks submitted:

D1-D6 - left lobe, submitted sequentially, from inferior to superior pole; D3-D4 are submitted following decalcification
D7-D9 - isthmus; D7-D8 are submitted following decalcification
D10-D15 - right lobe, submitted sequentially, from inferior to superior pole; D10 and D12 are submitted following decalcification

(E) (right central neck lymph node) Received in formalin is one yellow and black fibroadipose tissue measuring 1.5 x 0.9 x 0.4 cm. One potential lymph node. Specimen is submitted entirely in E1. [REDACTED]

Microscopic Description:

Complete microscopic evaluation has been performed. [REDACTED]

Appropriately reacting controls have been performed and evaluated for all stains on this case as required.

Histopathology has a list of IH antibodies that are regulated as analyte specific reagents (ASR's). These assays were developed and their performance characteristics determined by the Histopathology Laboratory in the Department of Pathology at The [REDACTED]

[REDACTED] They have not been cleared by the US Food and Drug Administration. The FDA has determined that such clearance or approval is not necessary for the ASR's. These tests are not investigational and are used in standard clinical care. In cases where

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Immunohistochemistry testing is performed, the following antibodies and their respective clones may be used to determine therapy for the patient: EGFR(31G7), ER(SP1), PR(1E2), Her2neu(4B5), CD117(Poly), CD20(L26).

Unless otherwise stated in the report, all tissue tested for ERPR by IHC, Her2 by IHC and/or HER2 by FISH have been fixed as per ANP.22998 for a minimum of 6 hours and a maximum of 48 hours.

ER, PR, Ki-67, p53 are reported as a semi-quantitative percentage of positively stained nuclei. Her-2/neu and EGFR are scored as follows: No staining at all is scored as (0), weak, incomplete membrane staining in any proportion of cells is scored as (1+), less than strong but complete staining in any proportion of cells or complete strong staining in less than 30% of cells is scored as (2+), and strong complete staining in more than 30% of cells is scored as (3+). All studies are performed on tissue fixed in 10% neutral buffered formalin and embedded in paraffin unless otherwise stated in the report.

Criteria	Yes	No
Diagnostic Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case (specie)	QUALIFIED	DISQUALIFIED
Reviewed by	Date Reviewed: 10/10/12	

Printed from: Default

End of Report

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